

ABNORMAL THOUGHTS OR BEHAVIOUR

Give urgent attention to the patient with abnormal thoughts or behaviour and any of:

- Sudden onset of abnormal thoughts or behaviour
- Recent onset of abnormal thoughts or behaviour

Management:

- If just had a fit →19.
- If aggressive/disruptive →84.
- If new sudden asymmetric weakness or numbness of face/arm/leg, difficulty speaking or visual disturbance: consider **stroke** or **TIA** →136.
- If recent head injury →18.
- If suicidal thoughts or plans ↗ 83.
- If difficulty breathing, respiratory rate > 30, oxygen saturation < 94% or oxygen saturation machine not available, give face mask oxygen.
- Check glucose: if < 3 or ≥ 11.1 ↗ 17 or if diabetes and < 4 ↗ 130.
- If thirst, dry mouth, poor skin turgor, sunken eyes, decreased urine: give **oral rehydration solution**. If unable to drink or BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- Consider involuntary admission if signs of mental illness and refuses treatment or admission and a danger to self, others, own reputation or financial interest/property ↗ 140.
- If HIV positive with recent positive cryptococcal antigen test, refer for urgent lumbar puncture (LP).
- Look for delirium, mania, psychosis, intoxication, withdrawal or poisoning and manage before referral:

Varying levels of consciousness over hours/days and temperature ≥ 38°C

Delirium likely
Give **ceftriaxone** 2g IV¹/IM.
Avoid injecting > 1g IM at one injection site.

Abnormally happy, energetic, talkative, irritable or reckless

Mania likely

Lack of insight with ≥ 1 of:
• Hallucinations (seeing/hearing things)
• Delusions (unusual/bizarre beliefs)
• Disorganised speech or behaviour

Psychosis likely

Dilated pupils, restlessness, paranoia, nausea, sweating or pulse ≥ 100, BP ≥ 140/90

Stimulant drug intoxication likely
If pulse irregular, chest pain or BP ≥ 140/90, do ECG and discuss with specialist or local poison helpline ↗ 178.

Smells of alcohol, slurred speech, incoordination, unsteady gait

Alcohol intoxication likely
• Give **thiamine** 100mg IV/IM.
• Give **sodium chloride 0.9%** 1L 6 hourly.
• Check for head injury.

Known alcohol/drug user who has stopped/reduced intake with tremor, sweating, nausea, severe restlessness/agitation or hallucinations

Alcohol/drug withdrawal likely
• If no other sedation given, give **diazepam** 10mg orally.
• If alcohol withdrawal, also give **thiamine** 100mg IV/IM and **oral rehydration solution**.
• If ≥ 8 hours since last alcohol, start alcohol detoxification programme →142.

Exposure via ingestion/inhalation/absorption of medication/unknown substance

Poisoning likely
Discuss urgently with specialist or local poison helpline ↗ 178.

Refer urgently unless:

- Patient with known schizophrenia who is otherwise well: give routine schizophrenia care ↗ 146.
- Patient with diabetes and low glucose, not on glicazide/insulin: if abnormal thoughts/behaviour resolve with dextrose, no need to refer, give routine diabetes care ↗ 130.
- Patient with known alcohol use who is otherwise well: if abnormal thoughts/behaviour resolve once sober, no need to refer ↗ 142.

Approach to the patient with abnormal thoughts or behaviour not needing urgent attention:

- If for at least 6 months ≥ 1 of: memory problems, disorientation, language difficulty, less able to cope with daily activities and work/social function: consider **dementia** →148.
- If unsure of diagnosis, refer for further assessment.

¹Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone.